

What are Eating Disorders?

Eating disorders are serious mental health conditions, that are characterised by extreme concerns about body weight, shape, image and/or eating. For people experiencing an eating disorder, self-worth is often very much caught up with weight, shape, and control over eating. These concerns lead to disordered and unhealthy patterns of behaviour including avoidance and/or restriction of food intake, strict dietary rules, vomiting, misusing medication for weight loss, and excessive or driven exercise. These behaviours can greatly affect a person's physical health, psychological and social functioning, and overall quality of life.

Approximately 10.5% of the Australian population have experienced an eating disorder at one point of their lives, according to Deloitte. Eating disorders can impact anyone, regardless of gender, age, socioeconomic status, cultural background, and body shape and size.

Eating disorders are not lifestyle choices, a "diet gone too far", or a phase. They are serious mental illnesses with some of the highest mortality rates from medical complications and suicide (See our handouts titled [Eating Disorders: What Are The Risks?](#) and [Starvation Syndrome](#)).

Recovery requires specialised treatment and support. With access to timely and evidence-based treatment, dedication and hard work, recovery is possible.

Below are five of the most common types of eating disorders that are recognised by the Diagnostic and Statistical Manual of Mental Disorders (DSM-5).

Bulimia Nervosa

Bulimia Nervosa is characterised by repeated episodes of binge eating, followed by compensatory behaviours.

- **Binge eating** involves eating a very large amount of food within a short period of time, and feeling out of control or unable to stop.
- **Compensatory behaviours** are ways of attempting to control weight or shape, and managing distress. This can include vomiting, misusing laxatives or diuretics, fasting, excessive exercise, or misusing over the counter, or prescription medications for the purpose of weight control.

Because of the large amount of food consumed in a binge, and the relative ineffectiveness of most compensatory behaviours, weight may fluctuate, stay the same, or may even gain weight.

The cycle of bingeing and purging can be out of control and lead to feelings of shame, guilt, and embarrassment. It can maintain preoccupation with eating, body image, and a fear or weight gain. This cycle is often kept secret and can go undetected.

Anorexia Nervosa

Anorexia Nervosa is characterised by **persistent restricted intake leading to significant weight loss and/or low body weight**. This is accompanied by an intense fear of weight gain, or persistent behaviour that interferes with necessary weight

gain.

People with Anorexia Nervosa often experience a distorted view of their body, believing that they are overweight when in fact they are dangerously underweight.

There are two subtypes of Anorexia Nervosa.

- **Restricting subtype** refers to individuals who severely restrict the amount and type of food they eat. They may also engage in other weight control behaviours such as excessive exercise.
- **Binge/purge subtype** also involves extreme restriction, but this is accompanied by episodes of binge eating and compensatory purging.

Binge Eating Disorder

Binge Eating Disorder is characterised by regular episodes of binge eating. Unlike Bulimia Nervosa, someone with Binge Eating Disorder will not engage in compensatory behaviours (such as vomiting, laxatives, fasting etc.).

Individuals with Binge Eating Disorder will often eat alone or in secret because of feelings of shame and guilt about their eating behaviours. Binge eating disorder can occur in individuals in a range of body sizes, and is often more prevalent in individuals seeking weight-loss treatment.

OSFED

Other Specified Feeding or Eating Disorder (OSFED) is when a person presents with some of the symptoms of other eating disorders (Anorexia Nervosa, Bulimia Nervosa or Binge Eating Disorder), but does not quite meet the full criteria.

OSFED is no less serious than other eating disorders and is the most commonly diagnosed eating disorder amongst adolescents and adults. It is also important to note that people can also have Atypical Anorexia Nervosa wherein they experience the same symptoms as Anorexia Nervosa, except that while eating a significantly low quantity and variety of food, they remain in medium or larger bodies. Please see our handout titled [Atypical Anorexia Nervosa](#) for more information.

ARFID

Avoidant/Restrictive Food Intake Disorder (ARFID) is when an individual struggles to obtain adequate nutrition, in the absence of the fear of weight gain and/or preoccupation with weight and shape that characterises AN, BN, or OSFED. In ARFID, feeding difficulties are either due to a lack of interest in food or lack of appetite, an aversion to certain sensory characteristics (like textures, taste, or smell), or feared consequences of eating such as choking or vomiting (not weight/shape based) which can lead to weight loss, nutritional deficiency, and difficulty maintaining a healthy weight. Please see our handout titled [Avoidant/Restrictive Food Intake Disorder](#) for more information.